



Dolores Jean Lavins Center for
Humanities in Medicine

The Tempest

Mayo Clinic Alix School of Medicine
Annual Creative Arts Publication



Dedicated to
Bran Ghislain

COVER: Transition
BY SHEILA MALEKIAN

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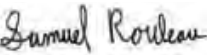
Foreword

Even though I’ve been a member of *The Tempest* staff for three years, I have always found the name a bit peculiar. The name refers to Shakespeare’s last play, which opens with a shipwrecking storm and subsequent plot that illustrates the storms we navigate and create within our lives. In addition, *The Tempest* pays homage to the founding of Mayo Clinic in the wake of a devastating tornado. The nature of a storm seems to be at odds with the nature of this creative arts publication, which provides a refuge for the free expression of students.

Storms are a constant in our lives, manifesting in unexpected and infinite ways – from the turmoil within to global chaos. The duration of 2020 and the first months of 2021 have been no different. The works within this volume are infused with the hurt and joy, the bliss and sorrow, the ennui and adventure of this past year. Each one immersed in the same yet separate storm.

Now, I have come to see *The Tempest* as an ideal namesake for this publication. Its essence is the sharing of experiences, as we all navigate through the storms inherent in our existence. To the contributors, thank you for your vulnerability and strength that grant us a glimpse into your reality – perhaps also granting us more insight into our own. It is my hope that *The Tempest* pushes us beyond seeing the “bad” or the “good” that can come with a storm, but encourages us all to be present within turmoil – to be embraced by the wind, shiver in the cold, and become the drops of rain that fall from our eyes.

With love,



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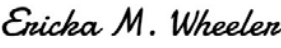
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Introduction

In this seventh volume of *The Tempest*, student contributors and editors once again offer creative, thoughtful, and moving images of individual experiences in medical school. And as in previous years, these unique portraits coalesce into a fascinating study of a community living the profoundly life-altering circumstance that is medical training. But this volume layers in the context of medical student life in the midst of a global pandemic, without the connectedness provided by a shared physical presence in the classroom, in a nation at times uncertain of itself. I found the reading of this volume to be highly impactful, with certain words and

images leaping off the page to engage me. Several pieces employ ironic undertones: “It’s going to sting a lot before we can turn it right side up” (Kent Richter); a photo of a subway car with a sign reminding absent passengers to keep a safe distance (Tomilona Ifelayo). Others send a more direct punch: “What makes the impossible possible?” (Valerie Melson), or “Everything is hard before it is easy” (Holly Thomson). Keldon Lin’s “Ode to Zoom” describes the now-universal experience of the pros and cons of conducting life by video. Maia Young voices a certain pain that we may not have been able to articulate on our own, “Did she ever

have the time to just grieve the loss of a part of herself? How do we do it?” And Shafiq Shaikh’s “lone falcon flying into a stormy winter night” is a poignant metaphor for the courage that is asked of these authors/poets/artists/photographers who are medical students in 2021, and who recently experienced the loss of one of their own. I’m grateful they have found the time to share themselves in these words and images.

Johanna Rian, Ph.D.
Program Director, Dolores Jean Lavins
Center for Humanities in Medicine

The Dolores Jean Lavins Center for Humanities in Medicine is proud to sponsor this this seventh volume of the Mayo Clinic Alix School of Medicine’s *The Tempest*. The mission of the Lavins Center is to support Mayo Clinic’s primary value, the needs of the patient come first, by integrating the arts and other expressions of human culture into the healing environment.

We have all lived through a most extraordinary year – one of pandemic, economic hardship, political unrest, racial injustice, social inequity, and ongoing damage to the planet we inhabit. We have witnessed challenges to the foundations of our democracy and how we are able to meet our obligations to each other at home, and on the global stage. We have all felt life’s existential angst marked by uncertainty, ambiguity, disconnection and a change in the experience of time and space of our existence. We have heard, repeatedly, how all of this has been described – without a sense of vision, imagination, and perspective – as “unprecedented.”

The events of our time are unprecedented only if we ignore the experiences of so many generations that preceded us, which we ignore at our own peril. The here and now is part of the ongoing narrative of the human condition. Because we are connected to the human family that was, we are also part of what will be. From our mistakes and failures, as well as our successes, we look ahead to create a narrative that future generations can look back on and take hope for their futures – one marked by visibility over invisibility, intellectual humility over hubris, connection over isolation, agency over dehumanization, community over chaos, flourishing over stagnation, meaning over lack of purpose and a commitment to our moral obligations to ourselves and others.

Those obligations hold all health care professionals and those in the field of health humanities to a high standard, one which requires us to embrace life’s ambiguity and our vulnerabilities in a way that allows us to be present to ourselves and those we serve.

New ways of learning in the very near future will require new ways of working together outside of compartmentalized, colonized ways of thinking. This is the remit of the health humanities in the future: a co-equal partner to the science of medicine that emphasizes the critical importance of imagination.

That imaginative commitment to what is and what can be is beautifully rendered in the work of our junior colleagues from the Mayo Clinic Alix School of Medicine in this issue of *The Tempest*. The tone of their work, as Dr. Rian has so carefully assessed, reflects a keen observation and an evolving sense of professional identity in an ambiguous and uncertain time. They are our teachers of and for the future, and we have much to learn from them. We are honored to have the opportunity to help foster their original work as presented herein.

Daniel Hall-Flavin, MD, MSc
Medical Director, Dolores Jean Lavins
Center for Humanities in Medicine

Head Study #1

BY PATRICK BRENNAN

Digital Painting



Snow Stop

BY ARCHNA PATEL



Frost

BY SHERRY ZHOU

The snow outside does give me pause
A rapid tumble, fluffy white
The roof now an embroidered piece
Of snowy strips and asphalt stitch.

A shadow swoops in, black as tar
Stands tall upon a former tree
A blink later and all that's left
A V-shaped trail upon the roof.

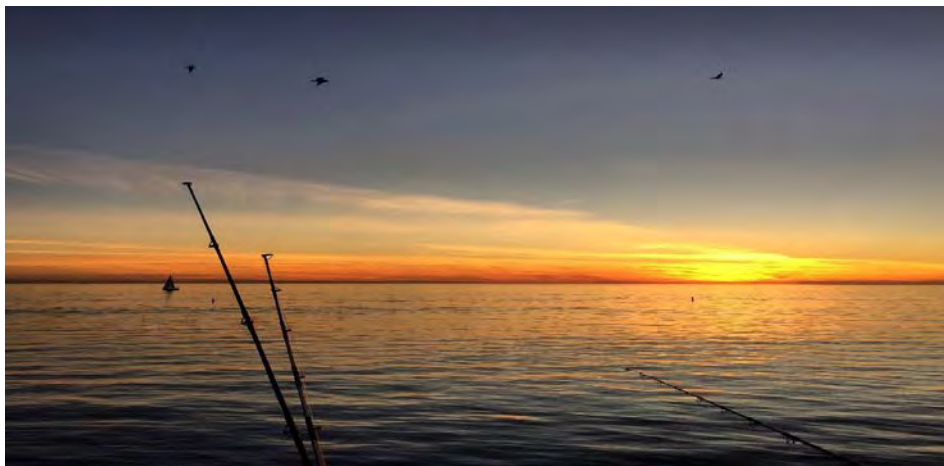
Colorful Canoes

BY JENNY YOON



California Sunset

BY SANTIAGO RESTREPO CASTILLO



Frontiersman

BY KENT RICHTER

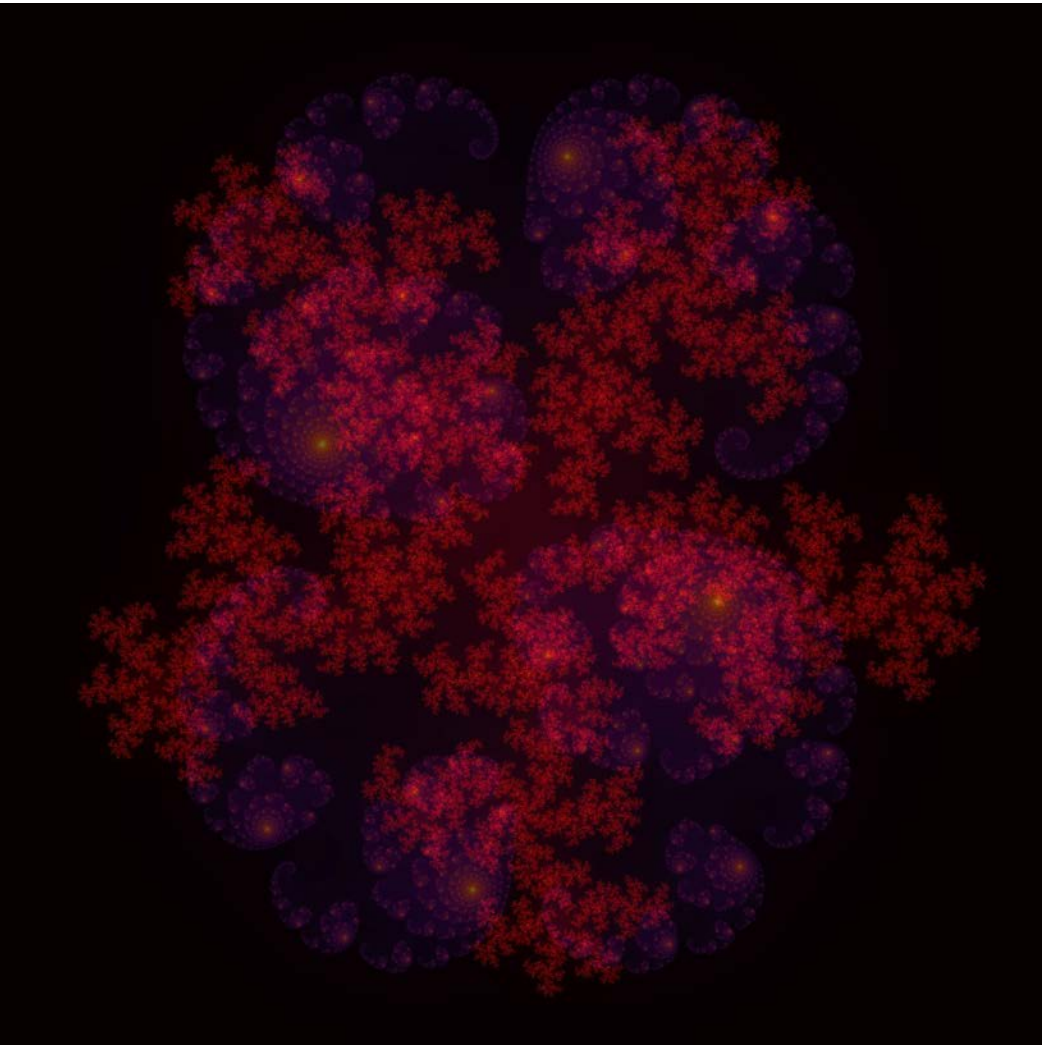
Describe to me times you happened upon the New.
A sound, a taste, a touch, a smell, a different hue.
Foreign. Virgin. Uncharted.
A thought, a word, a vision, a new *terra*.
Each discovery birthing new humanity: the founding of a new era.
A momentary golden age in the history of one's own senses.
A nascent neuron fires into space, proclaiming news just borne;
Awakening others, it further spreads, a new circuit trumpeting a new horn.
Connections made, bridges built, algorithms patterned,
From darkness comes light; from blindness, comes sight.
Ex-nihilo, symphonies sound from chaos; the bang of a new universe, yielding Eros.

And now, oh Man, where now do we look to experience the New?
What stone left to turn, what land yet to discover?
What frontier filled with wonder, what Atlantis to uncover?
Where once, we set sail in 1492, or landed with Apollo, leaping to the Moon,
Now we sit, we fume. Ever bothered, never moved.
How can one avoid such desire to deviate from "the same"?
To scan for seas uncharted, shores untainted, forests untamed?
Oh, to be the first who woke in morning dew,
Whose gustation bloomed from forbidden fruit.
The first to love, to laugh, to have tears blur their sight?
To wonder if death would come when the first sun sank into night?

Awake, oh Man, cast off the chains of monotony -
Cease idle speech, drowning life in egophony.
Seek meaning, write scripture, forge lore -
Be moved, risk all, explore.
It might mean death, but what a scaffolding!
It might mean ruin, yet delightfully challenging.
Live not to be seen, but rather to see;
Seek not to impress, but rather to be.
Let fall from your eyes the scales of pride, conceit, envy.
Boast not of your greatness: behold God's plenty.
Look for dreams which rise with leaven; turn not toward space, but to Heaven.

Moyamoya.py

BY DILEEP MONIE



Moya Moya

```
''' i seek support for the journey. '''
import numpy as alpha      # a Mayo legacy lays the foundation.
import matplotlib.pyplot as omega    # Hunter of the neural code reveals all.
%matplotlib inline

''' how will we function with diminishing resources? '''
def moya(forge, new, paths, around, all, obstacles):    # define our mission.
    conduit = alpha.linspace(-1.25, 1.25, forge)    # our field is expansive...
    narrows = len(conduit)    # yet constricting;
    hunger = alpha.linspace(-1.25, 1.25, new)    # wide open...
    ephemeral = len(hunger)    # yet fleeting.
    angiogenesis = alpha.zeros((narrows, ephemeral))    # the mind is blank;
    persist = complex(around, all)    # a constant limited only by imagination.

    ''' we must grow past the Rubicon and descend into chaos. '''
    for tem in range(narrows):    # perfuse...
        for pest in range(ephemeral):    # all branches.
            flow = complex(conduit[tem], hunger[pest])    # a serpentine bypass...
            guidewire = 0    # emerges from the void.
            for branch in range(paths):    # now dig deeper! find the key:
                flow = (flow**obstacles) + persist    # collateral circulation.
                guidewire += 1    # our advance is incremental...
                if (abs(flow) > 4):    # but have we gone too far?
                    break    # good enough. stop the cycle; move on...
            angiogenesis[tem,pest] = guidewire    # and note where we have been.

    ''' we have done all we can do:
    the storm subsides; equanimity remains. '''
    return angiogenesis    # reflect on our creation and absorb lessons learned.

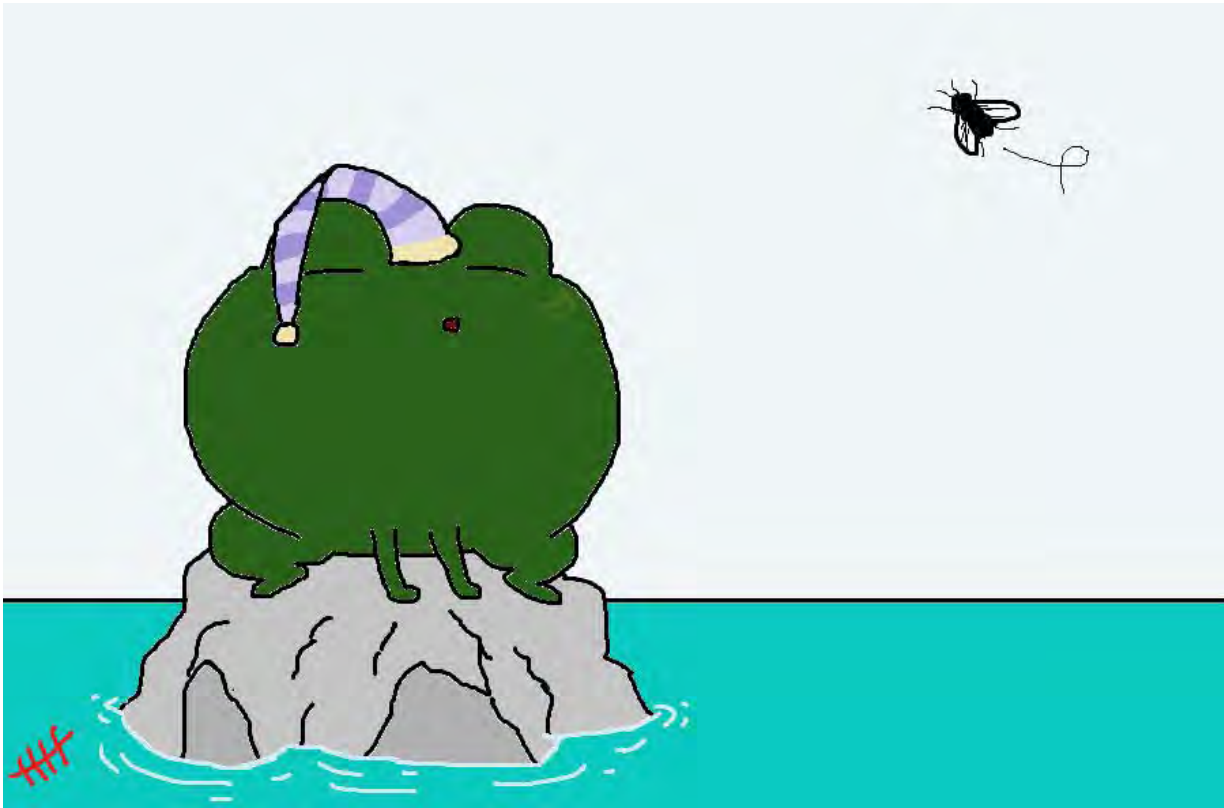
''' beginning from Fatou dust, we end in a puff of smoke. '''
look = moya(2048, 2048, 256, -0.42, -0.7, 5)    # set a vision...
again = moya(2048, 2048, 256, 0.285, 0.01, 2)    # for our operations.

''' at last, we glimpse the sanguine glow at the end of the vessel. '''
omega.imshow(look.T, cmap = "hot")    # our true colors...
omega.imshow(again.T, cmap = "inferno",    # burn bright...
             alpha = 0.4)    # and bleed through the matrix.
omega.axis("off")    # ignore all metrics of success; they are irrelevant.
omega.savefig("moyamoya.png",    # save our essence,
             dpi = 1200,    # imperfect, resolving...
             transparent = True)    # to be open and sincere.
```

Moyamoya is a disease in which stenosed internal carotid arteries cause cerebral hypoperfusion, inducing compensatory neovascularization that resembles a “puff of smoke” on imaging. This poem is composed of valid Python code that uses variables, comments, and logic related to the pathogenesis of this disease as a metaphor for perseverance. The libraries imported have rich ties to both Mayo Clinic and neuroscience, with NumPy authored by Mayo Graduate School alumnus Travis Oliphant and Matplotlib by electrocorticography researcher John Hunter. When run in a Jupyter Notebook, this program outputs superimposed fractals derived from the Julia and Fatou sets — an artistic interpretation of moyamoya.

Bean The Frog

BY HARVEY HUANG



A year and a half ago, I created a rather unconventional Instagram account, @bean_the_msfrog, in which I depict days in the life of Bean, a cheerful fictional frog who exists in the world of Microsoft Paint. Many of the illustrations have been reflections on my own state of mind.

Here is a recent doodle that captures my rocky transition from medical to graduate phase in the MD-PhD program. It is a quirky parody of the famous Daoist story, “Zhuang Zhou Dreams of Being a Butterfly.” There are three thematic parallels that I wanted to illustrate:

1. *Confusion*: What will my thesis project be? What will my career as a physician-scientist look like? Will I even succeed?
2. *Endless transformation*: From MD years to PhD years, back to MD years, and beyond, the role that I need to fill will never cease to change.
3. *Enlightenment*: I cannot predict how I will have grown by the end of my PhD, but I am optimistic that I will arrive on the other side a better thinker than I am now.

Living with Dysautonomia – What Standing Up Feels like to Me

BY RACHEL LOPDRUP



A nation on my shoulders

BY NEHA DEO

Failure was never an option for me.

Every time I fail
I am reminded that I have let my country down.

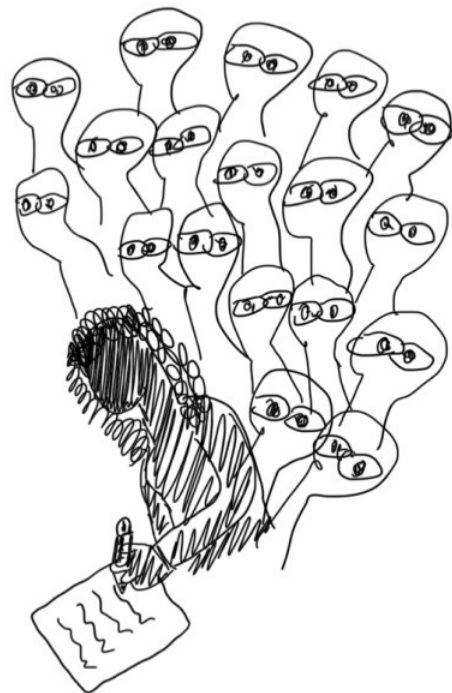
My ancestors, who worked as slaves in the sugarcane industry
on an unfamiliar island.

My grandfather, who passed before reaping the fruits of his labor
in an unfamiliar country.

My parents, who worked as immigrants to make ends meet
on an unfamiliar continent.

In failure I disappoint a community,
a country,
that rests on my shoulders.

My ancestors' blood runs through my body
& this serves as a reminder
that I cannot surrender.



Fragile

BY MAIA YOUNG

Growing up, we had no choice but to see the glass half full. The military in my dad didn't appreciate whining, and the Jamaica in my mom was simply not programmed to tolerate complaining. Whenever an outcome was not what my brother, sister, or I would have considered favorable, my parents would cheer us up by focusing on the positive, the silver lining, the glass half full. Admittedly, relatively good fortune throughout adolescence and young adulthood have made it rather easy for me to practice this habit on my own.

On one such lucky day in the fall of 2018, on which I was scheduled to volunteer as a patient advocate at Bellevue - a role I served in for two years while in college - I had been able to swap my usual 8-midnight shift to an 8-noon morning shift. Oh, happy day! Instead of spending my Friday night in the overcrowded, under-staffed Kips Bay emergency department, I could "get it over with" in the morning and go on about my weekend. This lucky morning even started out smoothly, with the trains arriving miraculously on time and my transfer to Penn Station not being complicated by the terrible awkwardness of walking by homeless New Yorkers without being able to provide spare change, which I never carried, or a hot meal, which I didn't have time for. (Eventually, I began to keep plain socks in my purse, which relieved my anxieties around being empty-handed in the face of those in need, although I could never be sure of the usefulness of these items to their new owners. Did it matter? Did I care?)

I felt even *luckier* when one of the first patients I assisted in urgent care was not only young, but also *regular*. Her regular height, regular attire and regular accent pleased my inner elitism; what a relief to not have to start my morning contorting to adjust to a "pt w irregular height," or exercising caution due to the lice and bed bugs that could be nestled in a patient's clothing or, heaven forbid, requesting a translator. The day was off to a regular start, and I secretly rejoiced. This beautiful and well-spoken black girl turned out to also be a beautiful and well-spoken artist who graduated from NYU and co-owned an art studio, where her pinky got caught in one of her machines more than a week ago, she explained to the resident attending to her. All of my internal praise of this patient's ordinariness and I hadn't even noticed the poorly wrapped and oddly colored bandage at the end of her slender, brown right hand.

She was just trying to put the finishing touches on an installation she was working on. Her pinky didn't even remain stuck for more than a second. As soon as she felt the rush of pain, she submerged the tiniest digit in ice-cold water and then applied some pressure and "shook it off" so she could return to her work. The resident listened, quietly nodding, as she recounted the story and her breath quickened as she got closer to explaining the bandage on her little finger. She soon lost sensation in her pinky, she said, and it started darkening and stiffening up and she knew enough from *Grey's Anatomy* about gangrene and doesn't think it's gangrene but what does she know, she's not the doctor, he is, so she just wrapped it to be safe and decided to finally come in because she has some time now.

Time.

The resident asked and she agreed to let him unwrap the bandage, which he did gingerly, bracing himself for the hardened, blackened fingertip waiting silently for its excision.

I would have also let all this time pass without any special attention. She's a reasonable and intelligent young woman, I thought, and I would have done the same things. I would have submerged my finger in ice, and moved on with my day, and not have given much more thought to the condition of the digit I only use to reach three keys on my laptop. Maybe if she had kept the finger on ice for more minutes, or elevated it at some point, or come into the urgent care sooner, her finger would never have shriveled up. Maybe if time were less of a commodity in her busy life - if she had the time to devote to the well-being of her *pinky* and not her life's dream - she'd still have it.

I thought about this young woman, tearfully protesting the necessary amputation of her pinky before calling her parents, assuaging them and herself: "It's only my pinky, it could have been worse. Could have been my *thumb* and then what would I do? I'll be okay. I'll be okay." Was she, too, conditioned to always see the glass half full? Were rainy days and negative emotions also forbidden in her household? Did she ever have the time to just grieve the loss of a part of herself? How do we do it? How do we, humans, in the eye of the hurricane and at the end of what we know, continue to see the glass half full when it shattered in an instant, leaving us no time at all to react?

The Upside Down - It's going to sting a lot before we can turn it right side up.

BY JAY VASILEV



Devil's Lake

BY ANJALI PANICKER



2020

BY OLUWATOMILONA IFELAYO



Unprecedented Times

BY KELDON LIN

This is only temporary
Need to flatten the curve
Back to normal soon

Right?

2.5 million lives
2.5 million dreams
2.5 million stories

Lost.

Healthcare workers and scientists
Cashiers, truckers, teachers
Countless others

Heroes.

Will it end?
Will we learn?
Will we grow?

Hope.

Jax Nights

BY MUHAMAD ALHAJ MOUSTAFA



Special Kind of Double

BY EMILY WU



Skin Deep

BY JACOB SMITH

Is confidence skin deep?
A mark here, a blemish there:
Imperfection in undulation.

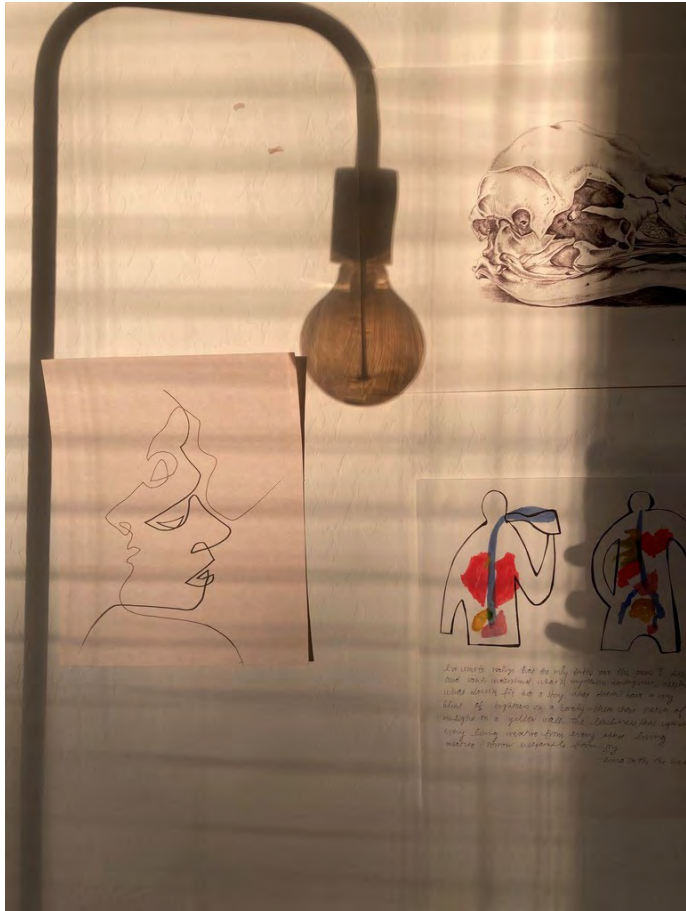
What do I trust?
A mass of flesh:
The body's creation.

Who stirs inside?
The self, the inner mind:
Humanity's voice aloud.

What remains?
A life built with stone:
Rugged, unbowed.

Wall Studies

BY EMILY WU



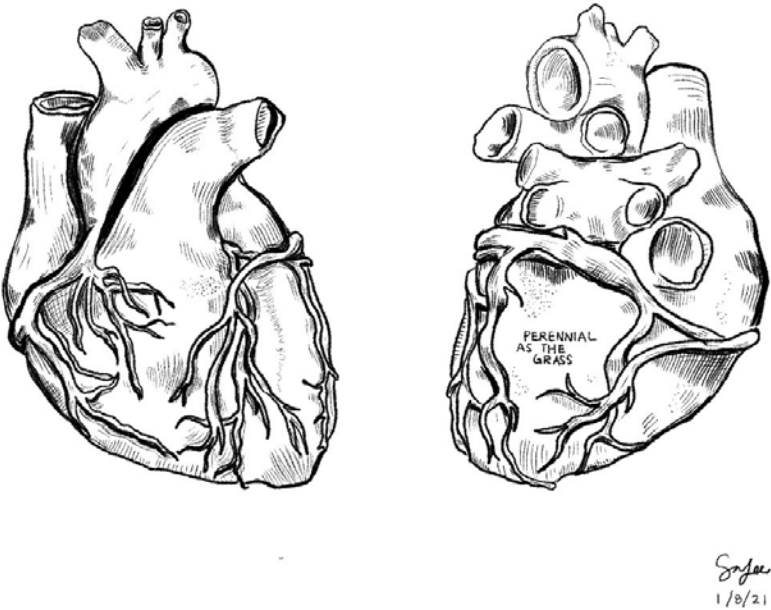
Lookout

BY REESE IMHOF



Perennial as the Grass

BY YEONSOO SARA LEE



Minnesota Lights

BY YVONNE BUI



Silver River

BY DAVID SORIANO

The silver fluid
Enough to flood the world
So reflective, painfully bright
Bodies lost, what's inside to replace
Mind and spirit become form

Now return from ignorance learned
Truth never heard over noise of steps and heartbeat
Like pounding drums, a dance of denial
So hypnotic a tone, the metronome

No longer you hide behind imagined walls
Now you perceive yourself
A traveler surveying the world
In the greatest mirror there is truth

Memories drown during travel
Stored dormant to be revealed

Soon the end of travel, the river's falls of life
Back to wonder and wander
Fall with the silver to your home

Taking Form

BY CORDELIA MCGEEHEE



The Danse Macabre

BY TORI RICCELLI

The Danse Macabre, also known as the 'Dance of Death' is a late medieval allegory which portrays the equalizing power of death. In many depictions, the living and dead dance together, with the dead leading the living to the grave. It is thought to have gained popularity around the time of the Black Death as a way to confront the ever-present prospect of mortality.

He whispers gently in the corner of every room
a shadow made of IV lines and drains and tubes
eyes old as the night and dark as the sea
splitting bone and soul, staring right at me.

I saw him reflected in the old woman's eyes
inviting her to *join us in the dance above the skies*
A gentleman, he leads, and soon the lady follows
He reminds her that her life is a gift that's merely borrowed.

He extends his hand to me, and I dance with him on earth
a crooked grin in hospice din - *it's not your time, it's hers.*
Push and pull as we waltz the room, whirling round and round her bed
give and take; lines drawn, remade; we know how this will end.

We tango down the unit hall, through one room and another
with enmity, our enemy, it is Death we strain to smother.
His slinking shadow swings to sounds heard from the monitor wall
as those we know - and those we don't - to his step freely fall.

But then I see a sallow face, with want of dancing partner
her life fulfilled and bones now chilled, she sails uncharted water.
And Death, he looks with sympathy and takes her hand in stride.
She joins him in the danse macabre, the dance above the skies.

I am from

BY VALERIE MELSON

music
like the consciousness of J. Cole,
the unapologetic realness of Jill Scott,
and the soul and grace of India.Aire.

I am from
PG County, MD
where suburb meets city,
where poverty meets privilege,
where minority majority rules.
We live in black and brown skin like nowhere else in this nation.

I am from
sayings like *hit dogs holler* and *Oh! Sugar Honey Ice Tea* when
you stub your toe because even when we curse, we must give
you a taste of southern hospitality —
All sweet and unassuming even when its underbelly is ugly.

I am from
smells like baked beans and baby back ribs.
Gumbo and seafood boils.
Fried catfish and homemade strawberry cakes on my birthday.
For my family, food is love.
When my grandmother gives you a plate, she’s telling you
that YOU are loved.

I am from
women who made a way out of no way
Who learned to walk when doctors said they never would
Who worked two or three jobs to make ends meet
Who wield education and perseverance
Who stood tall when oppression knocked on their doors
Who gave me the privilege of never having a need unmet.

I am from
a line of computer analysts, engineers,
beauticians, lawyers, military servicemen,
baggage handlers, seamstresses, and teachers.

I am from
hand-me-down shorts that I still haven’t grown into and stitches
of handmade prom dresses, a mix of baggy sweats and bodysuits.

I am from
problems like months with 11 people in one home.
Divorce and 1.5 hour commutes for decent education.
But my problems bring joys like 4 nieces and 3 nephews.
And opportunities I didn’t know to dream of...

I am from
teachings like education is the key to success even as
we’ve all seen degrees don’t stop bullets.
I learned early the importance of advocacy,
understanding, and community.
Learned to be hungry for change.

I am from
joy like my mother’s love, my friends’ understanding,
and the potential that youth brings.
Joy that allows me to choose a profession — To love
what I do.
And to *choose* every day to love the woman that I am
growing into just a little bit more.

I am not from

BY VALERIE MELSON

the streets.
I don’t know that struggle
Though my heart breaks for people who look *just* like me who do.
This cul-de-sac has conferred a level of protection I wish
wasn’t necessary.

I am not from
anger or unintelligence,
nor any other stereotype you thrust onto me.
Forcing you to see the complexities of me,
To see me
is exhausting.

Has it ever been exhausting
simply being you?

I am not from
Struggle.
Privilege now knows my name more so than hardship ever did.
Sometimes I try to hide truths from myself.
So here is one truth that I know, that I barely dare whisper...
For if my ears hear, then my heart may know that —
PG is starting to fit me awkwardly.
I often wonder who is changing faster?
It or me.

How do you claim loving a place into the best version of itself
while leaving it in the hands of others?

What does abandonment look like if not this?

Each mirror, a constant reminder of me—

the walking contradiction.

I am not from legacy,
There were no steps on this path for me to follow
I felt in the dark for the thing that I could not name but
knew was tangible
I felt lost and uncultured this whole way through
‘til I thought, if this is uncultured
Who robbed me of mine?
And then told the collective we, that ours was too
unkempt, too unapologetic, too ugly, too dark
We are the collective of our environment
That told us to be silent, to be less than, to be dead, to be
anything but the human beings we are.
Now we claim beauty in the parts of us that we refused to
make into your image
Our skin, this hair, our collective blackness
Fuel in my belly to claim —

EVERYTHING

There is no space that cannot be ours,
For when one life is cut short, we remind you how holy
the beat of blackness rings
How much faith exists in a people who will themselves
into continued existence.
No compass needed
We have learned to make our own way forward
To make the impossible possible.
What makes the impossible possible?
And am I enough?
Or will the impossible see my uncertainty and laugh.
Will I be David to its Goliath?
Or the ant on the bottom of its shoe...

Black or White: It is not Always that Simple

BY GABRIELLE CUMMINGS

Race is often one of the first things one notices about a person, whether it is conscious or subconscious. Whether one likes it or not, one immediately notices the tone of another person’s skin. I have always felt like an outsider in terms of race and cultural identity because I am both African American and Caucasian. However, I am thankful for this background because it allows me a unique perspective on race which has been especially beneficial in my medical education experiences thus far. In this piece, I want to share some of my personal experiences with race and help medical students navigate this intricate, but essential construct.

The first step to addressing race in medicine is to actually *address* it. We, as future physicians, need to ask patients about their race and background even if it makes us feel awkward. With time, the awkwardness will subside, but we need to make the effort to get to know our patients and where they come from because it is pertinent to their health. Race plays an important role in the preventative measures and lab work we choose and at what age. For example, according to the American Cancer Society, the age one should offer prostate cancer screening differs if the patient is an African American (2020). Those with average risk are offered screening at age 50, but African Americans are considered high risk and should be offered screening at age 45 (ibid.). Ethnic background also

plays an important role in the lab work we choose, such as hypertension and diabetes screening (Piper et al. 2014; American Diabetes Association 2002). Many physicians choose to screen for these diseases earlier and more frequently in African Americans. In my own experience as a patient, I have never had a doctor directly ask me about my race. As a result, I inherently assumed that race was not an important factor in my care. I saw the same primary care physician throughout college, and race was never mentioned. When I began to apply to medical school, I had to ask to see all of my medical records. To my surprise, I found that my medical records listed my race as “Hispanic,” and “Puerto Rican.” At the time, I thought it was laughable that they had looked at my light brown skin and assumed that I was Hispanic. However, upon learning about all of the preventative screening that varies based on race, I am now disappointed that my doctor did not bother to ask me about mine.

So, how does one bring up the topic of race? I have had plenty of experiences with how not to address race. While growing up in the south with ambiguous light brown skin, my classmates and peers were always curious about my race. They would frequently ask “What are you?” This question deeply disturbed me because I was a young girl just trying to figure out my identity and where I belonged. I did not see why knowing my race was important

and I definitely did not like the way the question was asked. However, there is one thing I commend my classmates for. They showed curiosity and had the nerve to ask about race! That being said, as future physicians, we can ask the question a bit more eloquently. When taking a patient’s history, we have to be curious and ask tough questions that may make us feel uncomfortable. First, take the time to build a rapport with the patient and gain their trust. I think it is important to be clear and concise when asking about race, while explaining how race plays into preventative medicine and promoting the patient’s health. From my perspective, it is better to avoid ambiguous statements, like “Tell me about your background” or “That’s a nice accent, where are you from?” Simply asking, “What race do you identify as?” is a better way to address the topic. Race is complicated enough, so I would try to avoid excess, unnecessary jargon when discussing it.

Now that we know information about our patient’s race, we have to check our judgments at the door. This means we do not impose our own biases on a patient because we discovered that they are African American. We all have biases and we need to become aware of them, so that we do not make assumptions based solely on a patient’s race. Race is part of our identity, but it is not *all* of our identity. It is great to be able to ask about race, but the curiosity does not stop there. We still need to understand the

social aspects of our patient, their access to healthcare, transportation and healthy foods. Knowing someone’s race does not tell the whole story. As a biracial female from a rougher neighborhood, people would assume that I was uneducated and I felt that I had to try even harder to prove them wrong. Patients should *never* have to prove themselves to their physicians. If we make no assumptions, then we are open and willing to learn and be surprised by our patient’s histories.

One may think that it is better to simply ignore race in medicine, believing that color blindness is what is needed to address race in medicine. I cannot count how many times someone has defiantly told me that they are not racist because they do not see race. Let’s be frank — we all see race, but it is how we choose to address it and our actions that matter. A friend once asked me why it was not okay to ignore race and I struggled to put the reasoning into words. I grew up spending a lot of time thinking about my biracial identity and wondering what my skin color insinuated to those that looked at me, but did not know me. It was an experience that I had difficulty verbalizing. Recently, however, I came across a great answer to the question of why color blindness is harmful. In a lecture on systemic racism, Edwin Lindo, JD argues that erasing someone’s contribution to society is the ultimate form of racism. He presents the example of the masterfully constructed

Egyptian pyramids and the myth that they were created by extraterrestrial aliens as a sign of erasing black people’s accomplishments from history. It is hard for society to conceptualize that black people could be capable of such skilled architectural abilities. In the same way, it is doing someone an injustice by denying their race and the struggles they have overcome to get where they are. In regards to the practice of medicine, doctors should inquire about the backgrounds of their patients to gain a clear picture of the circumstances patients overcame to get to this point in their lives. This allows you to provide care that is sensitive and personalized.

I personally believe that the tips outlined above are an effective approach to addressing race in medicine. We, as medical students and future physicians, should maintain curiosity, be explicit, and most importantly, be aware of our own biases that can harm the patients we intend to heal. I have been the girl at the intersection between black and white, which has given me a good idea of how complex race can be. Nonetheless, it is only as complicated as we make it. Future physicians need to be able to talk about race without feeling embarrassed. I hope that my story and these simple steps will help future physicians to embrace rather than fear discussing race with their patients.

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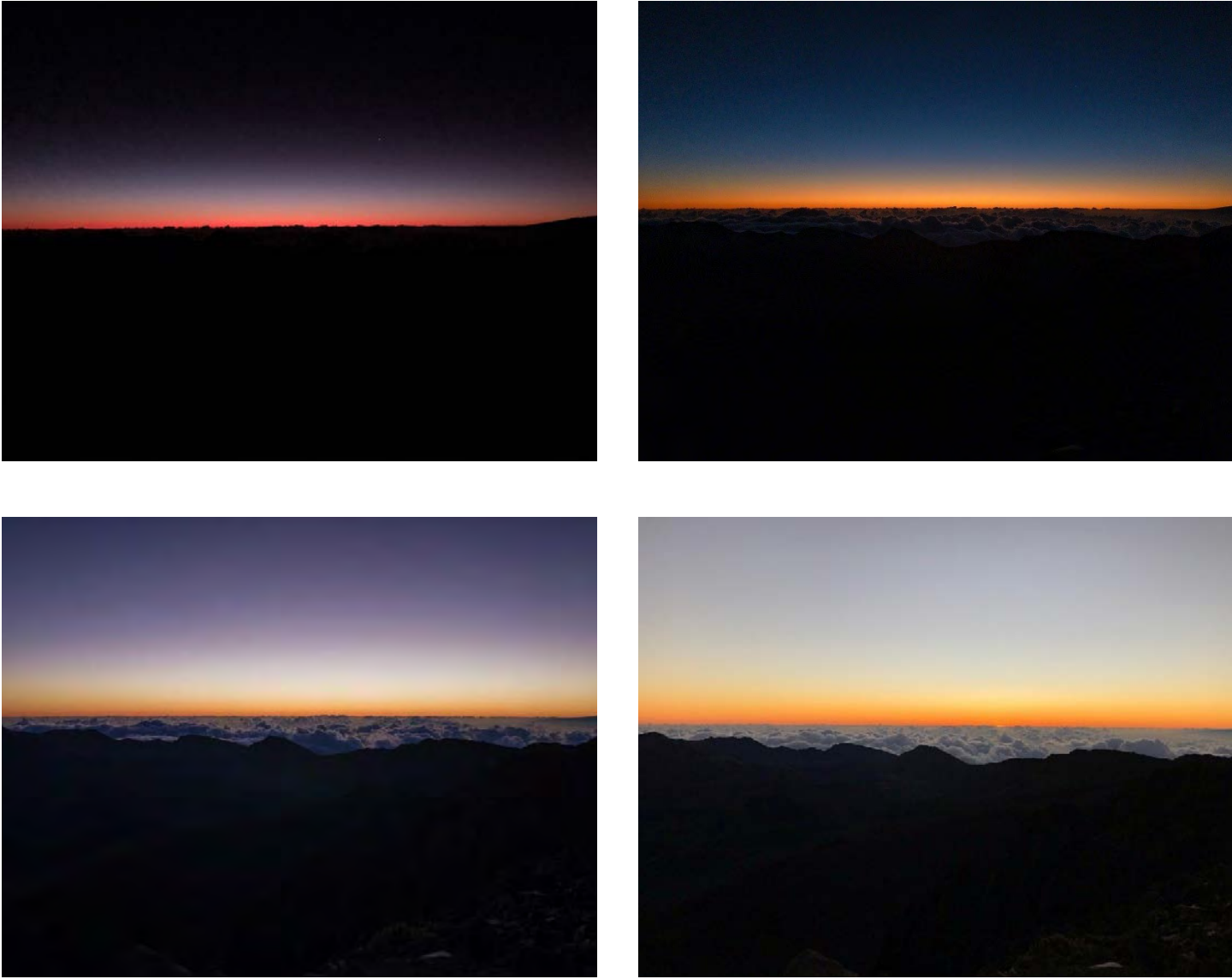
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Haleakala Sequence

BY SHERRY ZHOU



My Neighbor

BY BRANDON NGUYEN



I didn't know how to do anything

BY HOLLY THOMSON

During college, I studied abroad in Valparaiso, Chile, and I didn't know how to do anything. On a hot day in February I slipped into an ice cream shop, looking to partake in the Chilean passion for frozen desserts. My request for a scoop of chocolate was met with puzzlement – turns out, customers must pay before they order. My classes were at a university across town, and it took me a long time to figure out the public transportation system. Some days, I would leave three hours early and walk to avoid the fraught process of riding the bus. Local students knew which days were strike days to support the movement for free public university; I never figured out their system, and more than once walked into an empty classroom. I asked my host mama: where is the post office? “In the mall,” she said. I wandered around for a whole afternoon before I located the three flights of stairs behind an unmarked door that would take me there. Every day required rehearsal. Tomorrow I need to send a postcard, so today I will look up the word for stamp.

When I started my third year of medical school, everyone said: you're finally going to do what you came here to do, and you're going to love it.

Well, here is a small sample of questions I had: How do I write a good note in under three hours? Where is the nearest bathroom? I am on rounds/in the OR – am I even allowed to go to the bathroom? How do I page someone? Is that MY pager going off? How do I answer a page? How early is too early to page the GI fellow? How do I talk to my patients about code status?

About hospice? Can I sit in this chair? If my resident answers a question incorrectly, and I know the answer, can I say it? When does the cafeteria close? If they say I can go home, can I go home? Vent settings??? How much detail is too much detail to present for Dr. X? For Dr. Y? For Dr. Z? I am about to pass out in the OR – should I stay or should I go? Are heart murmurs a real thing that people hear? This patient is so sick – what am I missing? Why isn't anybody eating lunch? Above all: How can I demonstrate that I am curious and competent while also being invisible?

I didn't know how to do anything. Thank goodness I prioritized locating all the best bathrooms, because I often needed a place to cry silently for a few minutes. Not knowing how to do anything all day is hard. Contrary to popular prognostication, I mostly did not love my first year of rotations.

I heard the same narrative about the clinical years of medical school as I did about foreign study: it will be one of the most wonderful experiences of your life. It's a well-intended message, but one that makes me feel bad. If this is meant to be wonderful, why am I so tired? I'm not floating, I'm treading water.

My last days in Chile spanned a cold June, but I still went for ice cream a few days a week. Eventually I figured out the real move – passionfruit. The bus drivers learned my usual route and offered kind salutations as they dropped me off at the end of Avenida Libertad. I turned my dismay at arriving to a canceled class into leisurely sips at my favorite coffee shop. When people ask me these days

about my time in Chile, I talk about how wonderful and life-changing it was. But I try to be clear: ease had nothing to do with it. Rather, I learned about my own ability to adapt and to persevere.

Now at the end of medical school, I still don't know how to do anything (will I ever?). But I have moments when I float. I talk to the case manager as soon as their shift starts so that my patient can go home today. I sit in pretty much any chair without worrying that I am taking up too much space. When my resident corrects me I say “thank you” instead of “I'm sorry.” I need rehearsal, but I share difficult diagnoses with my patients without faltering. When my attending gives critical feedback on my presentation, I don't take it personally. Even if I suspect I will be able to leave early, I bring snacks. I teach a first-day fellow how to page. I hear a murmur of aortic stenosis (probably). I let the small triumphs add up into a steady feeling of warmth that can only be confidence, and that can, most days, block out the crippling doubt.

As I prepare for residency I repeat one sentence to myself, Goethe's simple truth: “Everything is hard before it is easy.” New experiences are not wonderful because they are easy; they are wonderful because they are hard, and they offer us the simple, powerful, life-affirming knowledge that we can do it.

An Ode To Zoom

BY KELDON LIN

Zoom, dear Zoom, how can I thank you?
Your brilliant blue hue and your breakout rooms, too
You always listen to me
Even when I'm on mute

It's been a whole year with you by my side
My camera, I promise, I will never hide
Only once have you left me:
When my laptop died

Your limitless backgrounds, both virtual and real
Your many viewing options, each one ideal
I can no longer hide
How you make me feel

You connect me with friends, regardless of distance
You are in every email; you have shown your persistence
My time belongs to Zoom
You are my existence

Your audio is flawless; your video is perfection
Even when I stumble and lose my connection
You are my North Star
You give my life direction

You're there when I sleep and when I arise
I express my love to you through my chatbox replies
Zoom, you are the window
That I always maximize

To you I am connected, with you I am in sync
With you in my life, I cannot help but think
“This is pure bliss”
When I click the “Join Meeting” link

My dear Zoom, a thousand ways have you blessed me
Are you my F5? Because Zoom, you refresh me
Whether dress pants or shorts
You always accept me

Little Bass Lake

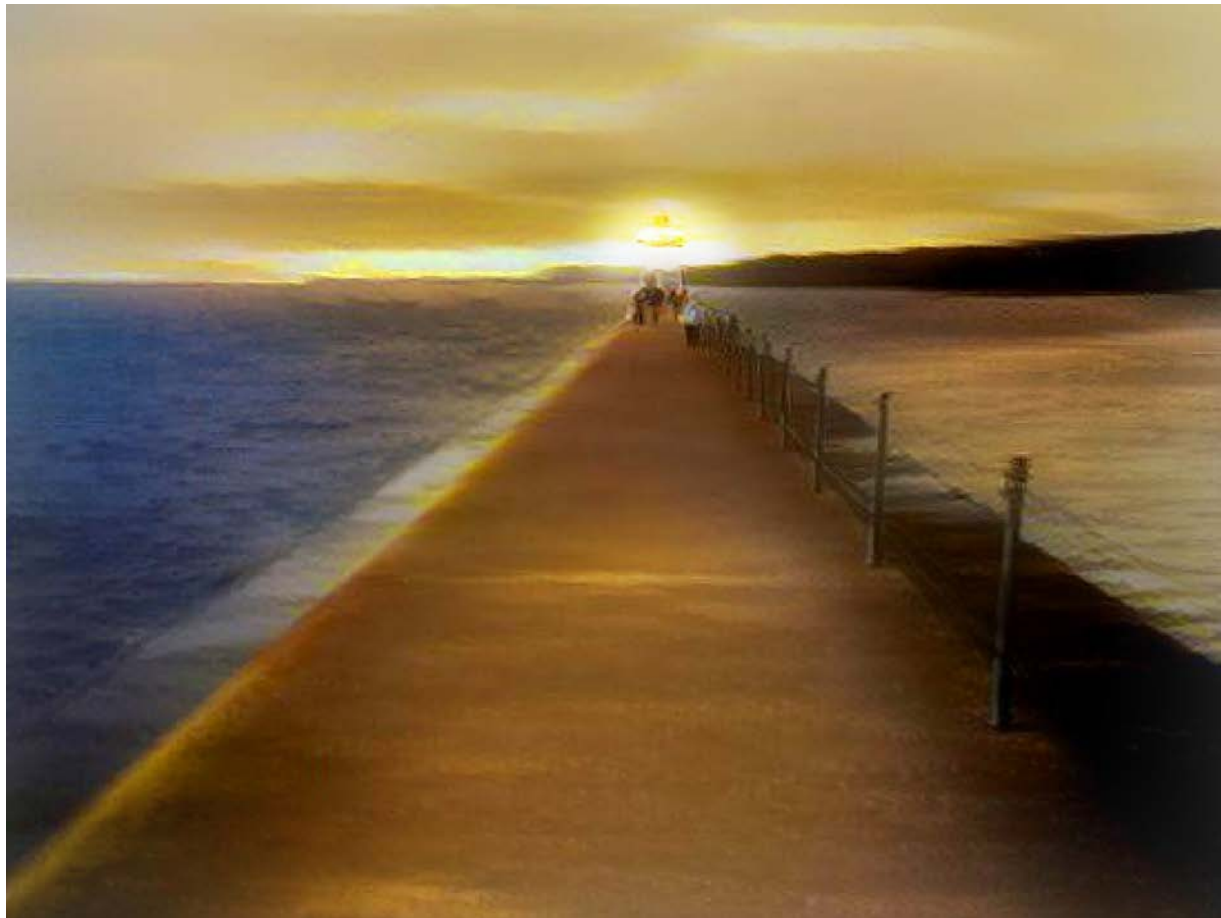
BY BEN NELSON



Path

BY ADIP BHARGAV

AI-assisted photography



because the battle isn't mine

BY MAIA YOUNG

each day
the cases
rise
as do
the sun
my hope
my anger
yet, as badly as i want to,
i can't seem to follow suit

who took my joy?

God knows

as my
to-do list grows,
my motivation withers
and i let blankets bury my bed-ridden body
until i rise, expel, consume, then slip
back into nothingness
until i'm too tired
to even sleep

which i fight
every morning
alarms in arm
hourly urgency

to be better,
work harder
produce something
if only good thoughts

apparently it's depression
that i bargain with every day
conjuring new deals and ultimatums
for a spark of the flame i hope is still there

sometimes we learn, in the hardest of ways,
that laying down is easy
and getting up is hard

but not impossible

The 7-Eleven that burned down on Bell Road

BY HENRY ZHENG

After the karate studio, there was the little Cuban café,
Where the fragrances of Havana spilled out onto the patio
Blending into the smell of cigar ash and gasoline.
Then there was the nail salon, whose matriarch sat at her desk
Taking only short breaks from her soaps to oversee her brood.
Finally atop a small hill was the 7-Eleven that edified my youth.

It didn't look like much from the outside.
Cigarette butts littered the stoop,
With chewing gum residue baked
Into the pavement like scattered pockmarks.
But upon entering, the fluorescent lighting and soft hum of refrigeration
Endowed the space with an electric quality.
From the hypnotically spinning hotdogs in their warmers
To the 32-ounce Monsters my prepubescent self was somehow able to purchase,
The 7-Eleven was a much-needed source of pure Americana.
The free Slurpees I consumed every July 11th were testaments to that.

After the fire, all that remained was twisted metal and molten plastic
Cordoned off by a few tenuous strands of caution tape.
Though there were whispers of foul play,
They faded quickly as construction began anew.
When I returned years later, I discovered the lot was now home to a Dutch Bros Coffee,
And the neighboring karate studio, café, and nail salon had similarly been replaced.
But while sipping on my cold brew,
I couldn't help but miss
The 7-Eleven that burned down on Bell Road.

Skies Aflame

BY NING MCKENZIE

Acrylic on Canvas



Red Socks and a Muted Television

BY RICHARD EBOKA

What would it take to walk in the shoes
Of a person bewildered, befuddled, confused?
We enter the room with our scrubs and our masks
Polysyllabic diagnoses – the bombast!
And let’s not even mention the white coat
That separates the patient – alienated, remote...
A lost lamb without a shepherd
The lack of eye-contact will the patient remember
How can we speak of therapeutic alliance
If human connection is reduced to “compliance”?

Your Astigmatism

BY DIANA HLA



Free Clinic

BY YONG-HUN KIM

Before cracks of sunlight split through the trees,
Alighting on a new day,
She stands outside the door
Receiving us,
Absent of complaints,
Of impatience,
Holding her two children
Close for warmth.

Maria, mid-30s,
Not demanding yet firm,
Intelligent,
Her eyes hardened by
Toys she can't buy for her kids,
Diabetes she can't control,
Never doing it the easy way,
Not that she had a choice.

Inside, offered resources to a food pantry,
She responds:
“Do they ask about immigration status?”

With one question
That I can't answer,
She slips from the hand
That tries to lift her up,
Not that she had a choice.

As she checks out at the front desk,
Her child looks at me
While holding mom's hand.
The hand that fed, bathed, and lifted her up.
I wonder
If she will do the same for her mother
When she has choices.

Tranquility in the Desert

BY NICHOLAS BERRY



Behind the Mask: A Reflection on Autoimmune Encephalitis

BY ERICKA WHEELER

“She’s been asking all morning for ‘the doctors,’” the 4th year Resident drawls slowly and sarcastically with air quotation marks as the workroom breaks out into howls and la-dee-da head-shakes.

“Well let’s go and see her then,” the Consultant’s deep tenor voice booms from his disproportionately prosperous belly as he nestles his notes under his damp, permanently sodden armpit. As if on cue, the makeshift professional team of seven - 3 half-starved residents, 2 sheepish medical students, 1 annoyed nurse, and the well-behaved, hairy expert - file out of the room.

“Finally,” you say, “The doctors are here.”

Seven people in long, white coats and plastic badges enter the room in a single-file line. Hospital-blue uniforms, clipboards large enough to cover their faces, masks covering their faces and teeth.... *Wait a minute*, you think. *Something’s not right*. You can’t put your finger on it, but you KNOW. *Something. Is. Not. Right*. You begin to feel afraid.

“I’m ready to go now,” you say quickly. “Unhook me from this thing,” you say expectantly, dragging the long clear tube attached to your shoulder into view.

The men in masks with clipboards seem uneasy. They loom over you with their fake plastic cards, and their staring inhuman eyes as if you’re the crazy one. You’re afraid.

“You can’t leave right now. You’re sick,” the belly-man says curtly behind his sheath of a mask.

You *knew* it.

“You’re not really doctors,” you blurt out. “You can’t make me stay here. Take this out, and let me go,” you demand as you pull out your phone. You figure if you record them, you’ll be safer. You can let the world know what happened once you’re safe from these imposters.

“Do you understand your brain is inflamed and if you go out there, you will harm yourself?” the portly one asks.

“Do *you* understand that *your* brain is inflamed,” you retort. Two can play this game.

They begin to come closer. Silence. You imagine their masks must cover needle-pointed teeth. You want to go home. Why won’t they let you go? You’re afraid.

Beamish Boy

BY DEBORAH MSEKELA



“You can’t be a nurse. You’re *fat*,” you spit out at the man who attempts to block your way to the door. *What do these people eat?* You think and then quickly dispose of the thought.

You’re afraid.

Two men in fake security uniforms arrive. They wear masks, too. They stare back, too. They block your way, too. “Help me!” you scream toward the medical student who has left the room, staring back at you in uneasy silence.

A sharp sting. Vision blurs. The wolves in masks still loom over you. They’re... blurry now. Sounds begin to dampen. Your right hand.... falls limp as you think, *Where did the real doctors go?*

The time of my life

BY ANDREW PINES

Whether due to turning 30, witnessing the deaths of strangers, or becoming excellent at time management, my sense of finitude has changed over years.

If you're peeved by the same stuff I am, your mind may crinkle when you hear the imperative, "think outside the box" (as it is often said bereft of an original phrasing to engender creativity, a definition for a thought as being inside or outside said box, and a conceptual appreciation for what a metaphysical box might be). Over the past four years, I've come to my own understanding of what thinking inside of a box means. It is a matter of binarity: right or left, right or wrong. Thought follows a line that may or may not divert at each distinction, forming a conceptual shape. Patient presents with xyz; differential is abc; treatment is 123. Xyw? Abd and 124. What do I know. A rationalist might respond that there is only one fact that cannot be doubted. But I nonetheless know a lot of things. Many of which are on the cusp of being forgotten, and many more that I have no memory of. Yes or no, I may not know, but I know the form of knowledge. Some of these ideas make their way to my heart and, through an obscure selection process, I keep them. A moment only means something in the future and each subsequent reflection will reform it. Time is not binary, neither by experience nor memory.

When we were beginning medical school, a friend commented that medicine is as risk-averse a career choice as one can find. We know we have the option for a well-respected career with a salary to comfortably support a family (our foreseeable career paths presented in the multiple-choice format of evaluations). Moreover, we know we are progressing. Even in day-to-day setbacks, we trend upwards, with our eyes over the horizon, towards abstraction. These are the days, my friends.

This day means something specific. 400 anki reviews, 40 practice questions, and 5 patient encounters. A quantified progression in knowledge, piece by piece, to complete a puzzle. One day, this will mean something qualitative. The hidden color in time will seep through, as all things seek beauty.

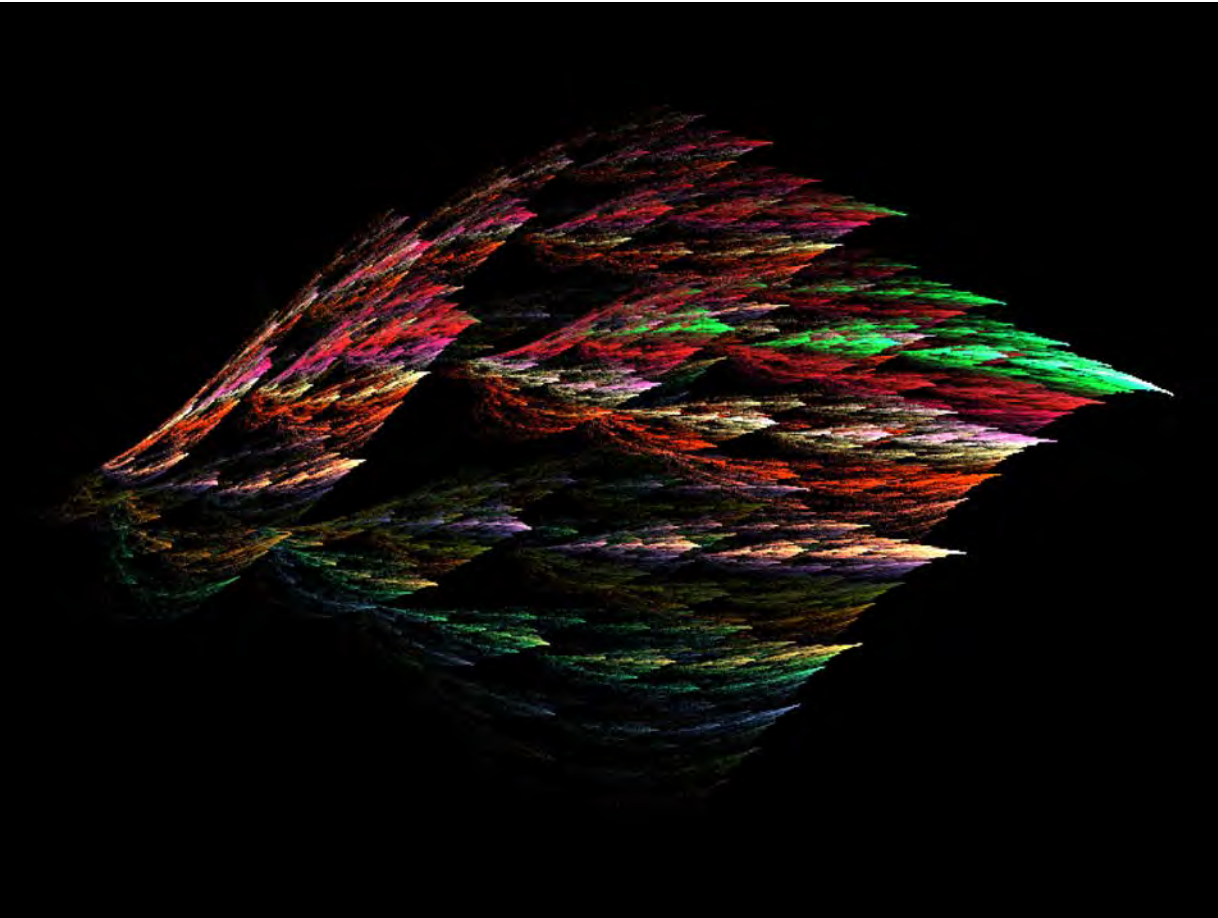
While awake I contemplate a flickering world and, through arithmetic, shape it to familiarity. While asleep all I encounter of myself is untamed. The time is out of joint. The definite article outlines a mirage, passing by, far above a sunken shipwreck. By the unnamed waves, a time flows forth regardless of intention. Thinking emits lightening, from end to end, blue to blue, a beginning enlightening.

Many of my classmates will soon become doctors. The fulfillment of a dream some of us have imagined for most of our lives. Beyond our past knowledge, beyond our imagination, for all of us - I hope for what dreams may come.

Differentiation

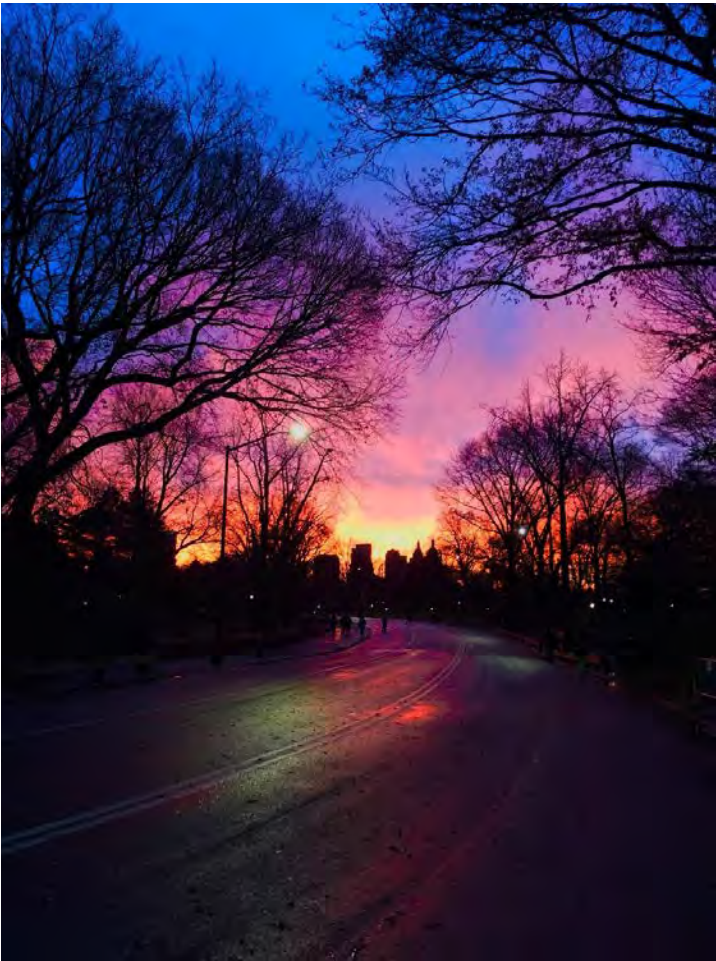
BY ADIP BHARGAV

Fractal-based digital illustration



A Walk to Reflect On

BY YVONNE BUI



Reunion

BY SAMI ABUJBARAH

Every time
I run into one
of my classmates
that I have not seen
in months I realize how
much time has passed by
and just how much I miss them
even if we never talked that much
a smile stays on my face for a while
but you can't really tell how happy I am
because of the mask

As a falcon flies

BY SHAFIQ SHAIKH

A lone falcon flies into a stormy winter night
Into the clap of thunder as it flashes a desperate silhouette struggling to stay aflight
Into the roar of sea as it rages underneath
Like a restless beast unsettled in its pit,
Promising in its black maw
Sudden violence of the void
A lone falcon flies into a stormy winter night

As the pall of dark falls thick and heavy
Enshrouding the horizon, unyielding and steady
What is it you reminisce about?
Your wings stretched into the clear blue sky?
Spiraling with the currents, as you soar through the heavens,
above your dominion of sweet delights?
Or perhaps to see with god's own eye
A rosy dawn setting the world alight
A lone falcon flies into a stormy winter night

As your battered body and broken wings
Flail against the fury of wild winds
What quiet promises do you make?
In the keen agony and the hiss of pain?
Warmth of your lover on a winter's night?
Your fledglings' first flight?
Or perhaps the sweet taste of bloodied guts glazed in silvery moonlight?
A lone falcon flies into a stormy winter night

As frigid cold seeps in from every crack in your skin
The glimmer in your eyes fading, and your resolve wavers and dims
What is it you set alight?
In the darkened corners of your forlorn mind
That reignites the fire in the furnace of desire
To bring the hallowed flame back to life
Which shines like a beacon ripping through the night sky
And paints a tiny spot on the distant horizon,
With the warm glow of a golden light
A lone falcon flies into a stormy winter night

Untitled

BY MARIAM KYARUNTS

When your lover tells you you’re the love of his life, it’s 5am and he just woke up from a restless dream. You had been awake the past hour staring at the ceiling, fearing the day. He spoons you and pulls your waist closer to whisper it in your ear. The warmth of his breath spreads through your body while the warmth of his words makes you feel like dancing in the clouds to the soundtrack of *La La Land*. Those words feel like the first leaf buds appearing in the dreary winter of your heart. They don’t belong there. You know that. Yet, when you hear him say that, you feel hope that maybe this could be the awakening you’ve been waiting for. Maybe this one will last. He’ll help you change into a different person and you guys can fight in the trenches together to accomplish your dreams. The feelings last a few seconds and dread inevitably takes over. You know that instead of the snow in your heart melting, it will just get pushed to the side of the road, only to solidify as a pile of vile dirt. A waste of space. A bother to everyone. You can never get rid of the qualities that make you you. He isn’t capable of dealing with that much dysfunction. He can only push the snow aside for now in a form of temporary salvation.

When he says you’re the love of his life, you think he is sincere. He probably means it in the moment. He thinks he means it because he doesn’t know you.

You want to pretend it’s the first time you’re hearing this from a man. But since you replay the tape of your pain daily, you remember all your wounds. The last time a man told you that you were the love of his life marked the beginning of the end. One by one, everything that he loved about you turned into a flaw unbearable to him. He used to say, “I love how unpredictable you are. Your constant changes keep me on my toes.” When my unpredictability involved not only spontaneous weekend trips and sexual adventures, but the throwing of plates, loud arguments, and throwing tantrums when he was busy at work, he became frustrated. He grew tired of having to excuse my absence every time I wasn’t present at a social engagement. He didn’t trust me after I maxed out his credit card to buy him an expensive watch. He hated when I got jealous and fought with him after he flirted with waitresses. He started saying, “Why do you always create chaos and act needy? Why can’t we just live a peaceful life? Why can’t you show up to spend time with my parents and friends? I show up for you. All I ask is that you show up for me too.” Another thing he used to say in the beginning was, “I love how uninhibited you are.” They all love it in the beginning. They love that I get so impulsive that I’ll let them do anything they want to my body just to feel something besides emptiness. Sex is better than cutting

right? What he doesn’t realize is that being uninhibited means I’m not just uninhibited with him. Whenever I was in that relationship and felt overwhelmed, I slept with strangers so I could disconnect from myself. “I wanna take care of you,” he used to say. He caught me cutting once and took upon himself the savior role. This sentiment changed when I threatened to kill myself if he went on a work trip without me. He started saying, “You aren’t healthy. You need help. You are too dependent on me.” As if I was an invalid. Someone who needed full time help just to survive. And after saying, “I can’t be your caretaker,” he left. My threats of suicide didn’t work anymore on him. “I can’t help you if you can’t help yourself,” he said as he returned a box with my possessions to never reach out again.

I probably deserved it after everything I put him through. So, I did what the only reasonable thing seemed at the time to someone who had just lost the main driving force of one’s life. I took a bunch of pills and called him. He put his finger down my throat and took me to the hospital. I spent three days in the hospital bed. Even the sound of construction didn’t get me up. I felt broken and alone. I lashed out at the doctors and nurses trying to take care of me. Then I told them what they wanted me to say so I could get out only to continue a downward spiral that took me a year to recover from. He changed his number and moved.

And after all these memories come back, you get out of bed. Turn on the lights.

“Get up. We need to talk.”

“Come back to bed. Let’s sleep more.”

“No we need to talk.”

“How about 30 more minutes?”

“No. I can’t do this anymore. We’re through. Get out of my apartment.”

The next day, you text him, “I miss you, let’s get back together,” hoping he forgives you one more time.

He is just another naive person with a hero complex trying to save me. He will crumble the moment he realizes that I’m not the harmless population inside the fort he’s trying to protect, but the force of destruction outside the gates. I am an unruly monster rampaging through the lives of people close to me, spreading my pain until everyone around me is as miserable as I am, and assaulting myself repeatedly in my effort to satisfy my hunger for stability, lifelong connections, love, peace. Please understand my innocence. I can’t control or tame the unruly force. The need to destroy arises without my control. Sometimes a small trigger or a trivial matter causes it. Oftentimes, it appears out of thin air. It then multiplies until I destroy other people or myself or both. I am simultaneously the mass of revolutionaries storming the palace and the queen they will execute by guillotine.

Author’s Note: This was a fictional piece I wrote when processing all the emotions expressed to me by my patients during my Psychiatry rotation.

Chemical Restraint

BY JON SUSSMAN

He says the thuds are softer with restraints in place
of the rain on the windows, leaving only their trace.

Same as the chemicals that flows through the air,
administered by our team, not in care;

as it moves through the tubing;
we're all aware it's his attitude we're lubing.

I can't remember if he couldn't sleep or was anxious,
but as my mentors remind me: autonomy is rapacious!

His nurse looked tired and to us that made sense
because earlier in the day on rounds he made us tense.

Then we said he was annoying
and we'd agreed to just keep re-deploying

our chemical restraints. Ativan.
1 mg IV Q4 PRN for anxiety. That's the plan.

Decisions

BY MICAELA WITTE



Polynices Interred

BY HENRY ZHENG

Atop each other’s shoulders we could see the world more clearly,

The wind whipping our hair as the bicycle zigged and zagged.

Through the dust and din we carved our way, until the wheels turned no more.

I can’t remember the last time we rode together.

It was years later I saw you,

A beanie pulled low to hide the patches of missing hair.

We reminisced – reliving those days spent basking in the light.

And still your sunny disposition never faded

Even as the cracks continued to widen under the weight of it all.

After I departed, you told me to hold you accountable

For your mind still raced even if you no longer spoke of growing old.

Dove Chocolate and Totoro,

They were her favorites you said.

Your brother called the day after Valentine’s.

If only we could still see the world through each other’s eyes.

Artists and Authors



Sam Rouleau is a fourth year medical student. He is constantly caught between his love of ice cream and his lactose intolerance.



Patricia Bai is a third year medical student at Mayo Arizona pursuing a career in ophthalmology. She was born and raised in Baltimore, MD and loves all things yarn-related, spending some of her favorite weekends knitting, weaving, and playing board games with friends.



Mylan Blomquist is a fourth year MD/PhD student based on the Scottsdale campus. She is interested in oncology and cancer biology. Mylan loves the desert, a good thrift store find, cats, and novels.



Andrea Collins is a third year medical student at Mayo Rochester who hopes to pursue pediatrics or psychiatry. She is from California and enjoys reading fiction, soapmaking and scrapbooking.



Noelle Driver is a fourth year medical student who plans to specialize in pain medicine. She started gardening during the pandemic and thereby enjoyed eating butternut squash throughout last winter.



Reese Imhof is a fourth year medical student at Mayo Rochester who is pursuing a career in Dermatology. Reese was born and raised in New York. Before coming to medical school, he had a career in graphic design and owned his own business. He enjoys spending time with his wife Nicole and son Eli, running with his dog Wally, biking, hiking, and photography.



Jon Sussman is a third year medical student at Mayo Clinic Alix School of Medicine. He attended The University of Chicago where he received degrees in neuroscience and the history, philosophy, and social studies of science and medicine. His academic interests include the philosophical questions of informed consent, the capacity for autonomy, the intellectual, cultural, and phenomenological history of mental illness, the science of healthcare delivery with an emphasis on continuous quality improvement and learning systems design, and the use of storytelling and narratives to make medicine a home and place of welcome for all.



Ericka Wheeler is a third year medical student from the Mississippi Delta who will be pursuing psychiatry. She enjoys spending time with her family, reading Octavia Butler, singing, practicing Taekwondo, and diagnosing personality disorders on reality TV.



Sami Abujbarah is a second year medical student at Mayo Arizona who likes to talk about his pet parakeet. He enjoys hiking in the dry, hot desert where there’s nothing to see, which is fine, because he’s often with excellent company. After years of science coursework and research, he’s finally trying to find his creative side and occasionally writes some poetry.



Adip Bhargav is from Ellicott City, Maryland but has bounced around from India to Wisconsin, Tallahassee and Atlanta. He is an avid tennis and ping-pong player and loves to write and draw. Adip is fascinated by the intricacies of the brain and the disease process of cancer. He hopes to find the intersection of these interests and incorporate them in his future medical practice.



Gabrielle Cummings is a second year medical student from South Carolina who hopes to specialize in psychiatry. When she has free time, she enjoys travelling, exercising, sleeping, and watching old and new TV sitcoms.



Rich Eboka is a second year medical student. He used to play Super Smash Bros., Pokemon, and watch a lot of anime. Now he just reads books and goes for the occasional hike, Arizona heat permitting.



Muhamad Alhaj Moustafa is a third year hematology oncology fellow and the current chief fellow at the Mayo Clinic in Florida. He is planning a career in lymphoma and is enrolled in the postgraduate master’s program. He is also the current Mayo Fellows Association Education Chair at the Florida Campus. He is originally from Syria where he did his medical school. Muhamad enjoys teaching and mentoring his Juniors. He is also an avid reader with a focus on books related to human history/origin/ cultures, as well as emotional intelligence, presentation, management, and leadership skills. He enjoys spending weekends with his wife and two kids.



Patrick Brennan is a second year medical student at Mayo Clinic in Rochester, Minnesota. Originally from Columbus, Ohio, he enjoyed studying oil, acrylic, and digital painting before coming to medical school. In his spare time, he enjoys hiking, skiing, grilling, movies with friends, and the Cheesy Gordita Crunch from Taco Bell. He is currently interested in neurology, internal medicine, and aerospace medicine.



Rebekah Dennison is a first year medical student at Mayo Rochester with interests in radiology, global health, and narrative medicine. She takes the “life” in work-life balance seriously, and can often be found playing violin, embroidering, painting, cooking, and/or attempting to learn Ukrainian. She grew up in St. Paul, Minnesota.



Harvey Huang is a third year M.D.-Ph.D. student from Vancouver, Canada. His research is in applying electrical brain stimulation to understand and modulate the human visual processing pathway. Clinically, he envisions a career in ophthalmology. In his free time, Harvey loves to sing, cook, read, brew coffee with his Aeropress, and powerlift (during non-pandemic times). You can find more on Bean’s life at @bean_the_msfrog (Instagram), and you can check out some of Harvey’s musical endeavors at harBops (Youtube).



Joe Barnett is a third year MD-PhD student on the Rochester campus. He is passionate about Psychiatry and is working on a PhD in Molecular Pharmacology and Experimental Therapeutics. He enjoys climbing and photography in his spare time.



Yvonne Bui is a medical student pursuing an MPH in Health and Social Behavior. In her free time, she enjoys exploring open-world games, sifting through recipes and food pictures online, and trying out new projects.



Neha Deo is a second year medical student at Mayo Rochester, originally from Vancouver (Canada) but born in Fiji. In her spare time, she enjoys eating Chick-Fil-A and tacos from Francis Tacos. She is still open-minded to any specialty but currently enjoys Emergency Medicine and Dermatology. She hopes to build a clinic and alleviate poverty in her motherland. She is also a hypebeast.



Oluwatomilona “Tomilona” Ifelayo is a fourth year medical student from Houston, TX pursuing Obstetrics and Gynecology. She enjoys creative writing, singing and songwriting, and working on her YouTube channel “The NotSoSmooth Road.”



Nick Berry is a third year medical student at the Arizona campus who enjoys playing golf and ping-pong, watching a variety of sports, and finding a nice hiking trail.



Nathan Chow is a second year medical student who still sometimes thinks he’s a first year. He wants to do internal medicine, family medicine, and emergency medicine all at once. During the day he spends his time cooking, reading, wishing Rich still played Smash, and can usually be found wondering about big questions like “is social media rotting our brains” while he plays with his food.



Mihai Dumbrava is a first year M.D.-Ph.D. student from Ontario, Canada. He is currently exploring new areas of medicine in an attempt to link his clinical and research interests in pediatric neuro-oncology. Outside of school, Mihai loves playing tennis, hiking, exploring brand new locations across the US, and has recently gotten into rock climbing.



Yong-hun Kim is a first year medical student at Mayo Clinic’s Rochester campus. He lived in the Bay Area, California for five years before moving to Minnesota, where he’s adjusting to the cold and slowly learning bits of Midwest culture. He discovered creative writing as a hobby during the pandemic and is grateful for the opportunity to share his work in the Tempest. His other hobbies include cooking and rock climbing.



Mariam Kyarunts is a third year medical student passionate about Psychiatry. She enjoys watching lowbrow reality shows, swimming, and consuming self-improvement content.



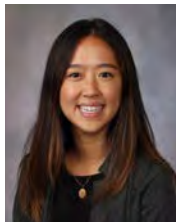
Sheila Malekian is a third year medical student dreaming of one day becoming an old-fashioned family doc who carries a doctor’s bag around and still makes home visits. Most of her time is spent attempting to carry her handicapped pup up and down stairs, murdering plants, and assembling 3D puzzles.



Dileep Monie is a fifth year MD-PhD student at Mayo Clinic in Rochester studying immunology and regenerative sciences, with a clinical interest in neuro-oncology. Outside of school, he enjoys urban freeriding, brewing beer, making things, and exploring the universe.



Anjali Panicker is a third-year Ph.D. candidate in the Immunology track at Mayo Clinic, Rochester, MN. Originally from Mumbai, India, she moved to Rockford, IL, to study antigen-specific immunosuppression. That stint led her to the Mayo Clinic. She is now exploring mechanisms by which specific antigens mediate transplant rejection. When she is not in the lab, you’ll find her swimming, painting, or hiking at one of the many Minnesota state parks or trails. She has learned to enjoy the midwestern winters. Of late, she has been learning West coast and single-time swing, and she is loving it!



Yeonsoo Sara Lee is a first year medical student at Mayo Clinic Alix School of Medicine on the Arizona campus, in the 2+2 program, who, like a pluripotent stem cell, is undifferentiated (and eager to grow and learn)! In a past life, she was an ecologist, and enjoys adventuring outdoors through hiking, backpacking, and climbing. She is passionate about women’s health, advocacy in medicine, and understanding/ combatting health inequities. She likes her roommate’s cat and farmer’s markets.



Ning McKenzie is a first year medical student at Mayo Arizona from Berkeley, California. She enjoys everything from backpacking to snuggling her cat, and is always being roped into new hobbies, like rock-climbing (pun intended). Ning’s other interests include changing her specialty of interest every 5 minutes, cooking for friends, playing board games, and trying to shield her plants from the Arizona heat.



Deborah Msekela is a fourth year graduate student from Tanzania studying the function and pharmacology of nicotinic ion channels. In her free time, she enjoys painting, reading and being outdoors.



Keldon Lin is originally from Rockford, Illinois and is now a first year medical student at Mayo Clinic Alix School of Medicine in Arizona. He is passionate about education, entrepreneurship, and technology and how these fields intersect with medicine. In his spare time, he enjoys playing basketball, working out, spending time outdoors, and watching movies.



Cordelia McGehee is an M.D.-Ph.D. student originally from the Twin Cities. Her Ph.D. studies and post-graduate work are in cancer pharmacology and mathematics. She is passionate about the integration of mathematics, biology, and medicine. Cordelia is an avid photographer in her free time and loves traveling to new places with her camera.



Ben Nelson is a third year medical student. He will be pursuing residency training in orthopedic surgery and will serve in the U.S. Army afterwards. In his spare time, he enjoys making the most of the long Minnesota winters by skiing, ice skating, and playing broomball. His current extracurricular obsession is chess, with hopes of continuing his winning streak against his brothers.



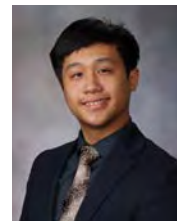
Archna Patel is a fourth year medical student originally from Central California. She completed an MPH last year on the east coast and is hoping to pursue Internal Medicine. She’s an avid consumer of TV & films, loves cooking elaborate dinners for no reason at all, and walking around her very suburban Rochester neighborhood with her friends when the sun’s out.



Rachel Lopdrup is a third year medical student on the Rochester campus. She is passionate about primary care and innovation in healthcare. Her hobbies include trying new art forms without looking up instructions, attempting a new food dish every week, trying new foods (preferably in new cities), and snuggling with her two rescue cats.



Valerie Melson is a second year medical student from Prince George’s County, MD who hopes to specialize in Obstetrics and Gynecology. She is passionate about social justice, community engagement, and Public Health. When medical school permits, she enjoys spending time with her family, baking, and traveling.



Brandon Nguyen is a first year medical student at Mayo Arizona from Honolulu, Hawaii—but don’t ask him if he knows how to surf. In his free time, he enjoys digital art, swimming, anime, cello, playing video and board games with friends, cooking, and baking (admittedly, he mostly makes cheesecakes). He adores Ghibli and ATLA references and is currently in a one-sided relationship with neurological surgery. And although his cheesecake-making skills are great, he has a lot to learn before he’s ready to save anyone.



Andrew Pines is a fourth year medical student who is pursuing (and is at this point quite close to catching) residency in psychiatry. Current research interests are neural circuitry, functional imaging, and biological correlates of psychiatric illnesses.



Santiago Restrepo-Castillo is a Ph.D. candidate in Virology and Gene Therapy working on mitochondrial genome engineering for human gene therapy applications. While he appreciates the practicality of CRISPR, he is a huge TALENs advocate. Some of the activities Santiago enjoys outside of school include writing, reading, playing the piano, and taking pictures.



David Soriano is a first year medical student on the Rochester Campus with the intent of pursuing a career as an orthopedic surgeon. He has several research interests within the field of regenerative medicine and medical innovation. He is an avid powerlifter, motorcycle rider, hiker, and tinkerer/DIY enthusiast in his free time. Through the influence of our lead Tempest editor and friend, Sam Rouleau, he is attempting to add poetry to the list of activities. He will leave that up to the critics.



Emily Wu is a second year medical student at Mayo Arizona, with interests in palliative care, geriatrics, and working with underserved populations. She finds it strange to write about herself in the third person but enjoys learning TikTok dances, coffee shops, and thrifting.



Henry Zheng is a second year medical student from Phoenix, Arizona. In his free time, he likes to watch movies and try new restaurants.



Tori Riccelli is a third year medical student at Mayo Rochester. Originally from Syracuse, NY, she fell in love with neural circuitry after studying the sensorimotor system in zebra finches. Her favorite activities include getting lost on runs, hanging out with her hamster Donut, attempting recipes way beyond her skill level, making pretty graphs with seaborn, and commenting on the weather at every possible opportunity.



Holly Thomson is a fourth year medical student from Tucson, Arizona matching into internal medicine this spring. She feels most like herself when baking, and in the past month has made: marsala cake, flourless chocolate cake, black tea snacking cake, a maple pecan tart, raspberry white chocolate scones, butterscotch pie, and peanut butter brownies.



Jenny Joo Hee Yoon is a third year medical student at Mayo Clinic Rochester from New Jersey. She adores working with the geriatric population and is very curious to see what type of doctor she will be in the future. Her hobbies include practicing yoga, listening to podcasts, camping, kayaking, and eating other people's food.



Sherry Zhou is a first year M.D.-Ph.D. student on the Rochester campus with an interest in medical oncology and the biochemistry and chemical biology of tumorigenesis. She grew up in Vancouver, Canada, and has also lived in the Bay Area of California and in Cambridge, MA before coming to Rochester, MN. She is passionate about sustainability and working with underprivileged and underserved communities. She enjoys reading, creative writing (especially free verse poetry and haiku), baking and cooking, playing piano, and any outdoors sport or activity.



Kent Richter is a fourth year medical student at Mayo Arizona pursuing neurosurgery. He is married and is the proud father of 3 daughters. When not studying for school, Kent spends his time with family going on walks, bike rides, hiking, or camping. He also enjoys landscape design and reading.



Dzhuliyen (Jay) Vasilev is a first year medical student at Mayo Arizona from St. Louis, Missouri. He enjoys blasting reggaeton in the shower and wants to learn how to fly a plane. He also got locked in the bathroom during anatomy block, missed his dissection, and was left without a bathroom door for over a month. He likes taking pictures, using cheesy quotes, and getting lost around the world.



Maia Young enjoys daydreaming about a future as a reproductive endocrinologist and tries to appreciate every experience on her journey toward that goal. As a previous “Medicine, Literature and Society” major at Columbia and hopeful member of the “Humanities in Medicine” distinction cohort here at Mayo, Maia does her best to encourage, empower and enliven herself and others through the intimacy of storytelling and the power of creating space for those around her. She hopes to harness the impact of medicine, specifically fertility, for women who would otherwise not have access to elective services and for whom the only path to upward mobility may be motherhood. Weaving, reading and plant-keeping have kept Maia sane throughout this pandemic, which has coincided with her first year in the MD program as a 2+2 student.



Shafiq Shaikh is a third year graduate student who works on RNA modifications. In his free time, you can find him reading anything from classic Russian literature to psychology or frantically turning pages from his favourite Shonen Jump manga. He also enjoys cooking, writing, playing tennis, training Jiu Jitsu and hiking/biking in the summer.



Micaela Witte is a third year medical student at Mayo Rochester, who is originally from Rochester, MN. She will be pursuing a residency in pediatrics. In her spare time, she enjoys running, watching the Green Bay Packers, playing with her puppy, and playing board games with her husband and friends.

Big Sur Sunset

BY JOE BARNETT



